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Correction of errors of substance may be made only on the floor of the House with the permission of the Speaker. However, correction of typographical or grammatical errors which Members suggest for the Bound Volumes which will be compiled at the end of the Meeting may be clearly marked in the Daily Report, and the copy containing the corrections submitted at the Editor's Office, Parliament House, not later than four clear days after the publication of the Daily Report.

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THE
PARLIAMENT OF THE REPUBLIC
OF GHANA

FIRST MEETING, 2026

Wednesday, 4th February, 2026

*The House met at the Chamber,
Parliament House, at 2:25 p.m.*

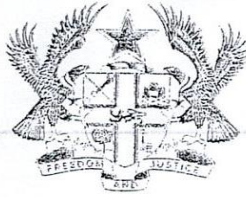
[THE SPEAKER, RT HON ALBAN
SUMANA KINGSFORD BAGBIN, IN
THE CHAIR]

[PRAYERS]

The Speaker: Hon Members, we would skip the item numbered 2 and take the item numbered 3. I have a message from His Excellency the President. The message reads:

**COMMUNICATION FROM THE
PRESIDENT**

Absence from Ghana



REPUBLIC OF GHANA

Jubilee House

The President

30th January, 2026

Rt. Hon. Speaker,

ABSENCE FROM GHANA

In accordance with Article 59 of the 1992 Constitution of the Republic of Ghana, I write to formally notify you of my intention to travel outside the country on 1st February, 2026, to Dubai, UAE, to participate in the World Governments Summit 2026.

Following this engagement, I will proceed to Lusaka, Zambia, for a State Visit from 4th to 6th February 2026. I am expected to return to Ghana on the evening of 6th February, 2026.

In my absence, the Vice President, Professor Naana Jane Opoku-Agyemang, shall perform my functions in accordance with Article 60 (8) of the Constitution.


JOHN DRAMANI MAHAMA

**THE RT. HON. SPEAKER
 OFFICE OF PARLIAMENT
 PARLIAMENT HOUSE
 ACCRA**

(2) Clerk/Usher
 B.U. to be read to
 the House.
 Jm 3/2/26

cc: The Vice President
 Jubilee House
 Accra

The Chief of Staff
 Jubilee House
 Accra

The Speaker: Hon Members, on your behalf, I want to acknowledge the presence of some visitors and these are students from two schools. The first, we have 123 students from Amanfopong Presbyterian Basic School. They are from the Asikuma Odoben Brakwa Constituency. I do not know whether they have changed the name, because I can see it written as Asikuma Odoben. But we used to refer to it as “AOB”. Is it still the same or it has been changed?

Alhaji Kobina Ghansah: Mr Speaker—

The Speaker: It is still “AOB”. Yes, I know the area; I have visited and campaigned there before. Is it still the same?

Alhaji Ghansah: —[Inaudible]—

The Speaker: Do you now represent them?

Alhaji Ghansah: Yes—

The Speaker: They are led by the headteacher of the school, in the person of Richard Annobil. I do not know whether there is a spillover, but to my left at the Public Gallery, you can see the young men and women there led by their tutors.

On your behalf, I warmly welcome them to Parliament and encourage them to keep on coming. I hope they learn something today from this House. They are welcome and they can kindly resume their seats.

[Pupils of Amanfopong Presbyterian Basic School were acknowledged.]

The Speaker: We also have 40 students from the Ketu North Constituency, and the school is Dzodze-Penyi Senior High School in Dzodze. They are led by the form master in the person of Oliver Amedume. They are seated straight ahead of me at the Public Gallery. On behalf of the House, I also warmly welcome you to Parliament and wish you all the best. Please try and learn something before they leave. Kindly resume your seats.

[Students of Dzodze-Penyi Senior High School were acknowledged.]

The Speaker: We will proceed to the next item and that is the Correction of *Votes and Proceedings* and the *Official Report*.

VOTES AND PROCEEDINGS AND THE OFFICIAL REPORT

The Speaker: Hon Members, we would start with the Correction of *Votes and Proceedings* of Tuesday, 3rd February 2026.

Page 1...6—

2:35 p.m.

Mr Frank Afriyie: Mr Speaker, thank you so much. With respect, may I bring a minor observation to your kind attention for correction.

Mr Speaker, item 4, number 8, the Minister for Roads and Highways, as always, was present here yesterday, but he has been captured under that column as absent. I pray dearly that the Clerks-

at-the-Table take note and correct accordingly.

The Speaker: Clerks-at-the-Table, kindly take note of that.

Page 7—

Alhaji Habib Iddrisu: Mr Speaker, on page 7, the item numbered 30, I was present yesterday, but my name is captured as absent.

The Speaker: Table, kindly take note.

Page 8—

Mr Vincent Ekow Assafuah: Mr Speaker, item number 7, the second line, or if I may even have to read the whole paragraph;

“The Rt Hon Speaker welcomed Hon Members to the First Meeting of the Session and expressed profound gratitude to God for guiding the House through the First Session...”

Mr Speaker, if we read the sentence, we can see that the Session could have been preceded with “Second”, that is the “Second Session”. In context, it may not be suicidal, but for purposes of accuracy, it would be proper that we bring in the “Second Session”.

The Speaker: Let me direct the Table to take note of the issue that has been raised by the Hon Vincent Ekow Assafuah.

Mr Kwabena Okyere Darko-Mensah: Page 6, the item numbered 20. I have been marked absent, but I was here yesterday. So, if it can be corrected, I would be grateful.

The Speaker: You said page 6, the item numbered 20 but page 6 has no item numbered 20.

Mr Darko-Mensah: It has. Page 6, under the sub-heading “absent”.

The Speaker: That is item numbered 4.

Mr Darko-Mensah: Yes, the item numbered 4.

The Speaker: Yes, but you said the item numbered 20. Item 4, number 20.

Mr Darko-Mensah: Yes, please.

The Speaker: Table, kindly take note of that.

Yes, Hon Abed-Nego Lamangin Bandim?

Dr Abed-Nego Lamangin Bandim: Mr Speaker, on the same page 6, the item numbered 15. I was here, but I am surprised to see that I have been marked absent.

The Speaker: That is item 4, number 15, not item 15. Table can take note.

Page 9...11

Hon Members, the *Votes and Proceedings* of the First Sitting, dated Tuesday, 3rd February, 2026, as

corrected, is adopted as a true record of proceedings.

Hon Members, we take the item numbered 6 because I do not have a copy of the *Official Report*. We will take the item numbered 6, Statements.

First Deputy Speaker, have you admitted any Statements?

Mr Bernard Ahiafor: Mr Speaker, yes, and it is indicated on the *Order Paper*. Item numbered 6.

The Speaker: There is none with me. I do not have a copy of the Statement. We have two then.

Mr Ahiafor: That is so.

The Speaker: All right.

Hon Members, Statements are usually handled by the First Deputy Speaker, so refer them to him for his consideration. He admits them, and my expectation is that he will always be handling them but once I am in the Chair, I will start before he takes over from me. We have two Statements. One standing in the name of the Hon Member for Ablekuma North, Hon Ewurabena Aubynn and the other, the Hon Member for Abuakwa South, Hon Dr Kingsley Agyemang. I think both are ceremonial speeches. They are ceremonial Statements. One on the occasion of World Cancer Day, and the other on the occasion of the 61st anniversary of the passing of Dr Joseph Kwame Kyeretwie Boakye Danquah. There is a third one, which has just been handed over to me, but I do not see that it has been admitted. So, we will leave that one.

First, we will take the Statement by the Hon Member of Parliament for Ablekuma North, Hon Ewurabena Aubynn, on the occasion of the 2026 World Cancer Day.

Please, Hon Member, you may do so now.

Sorry. Yes, Hon Minority Chief Whip?

Mr Frank Annoh-Dompreeh: Mr Speaker, if you may indulge me, you have counselled us that we should try and apply ourselves to the rules. Order 93(8) requires the Clerk to make copies of these admitted Statements to Hon Members. Yes, for the avoidance of doubt, if you will permit me, I read, “The Clerk shall make available to Members, copies of admitted Statements.” It is on the console—I see. Mr Speaker, I am guided. I have been told that electronic copies are on the console.

2:45 p.m.

The Speaker: Those are some of the challenges of transition. Please, always consult your console to see whether it is there or not. Hon Member, you may do so now.

STATEMENTS

World Cancer Day 2026

Ms Ewurabena Aubynn (NDC — Ablekuma North): Thank you, Mr Speaker, for the opportunity. Today, I rise on behalf of the people of Ablekuma North, and in solidarity with millions

across Ghana and the world, to mark World Cancer Day 2026, observed under the global theme, “United by Unique.” A theme that reminds us that behind every statistic is a human being, a family, and a community whose lives are profoundly affected by cancer.

Mr Speaker, cancer is no longer a distant or rare disease in Ghana. It is a growing public health challenge that cuts across age, gender, income level, and geography. According to available data from our health authorities and cancer registries, Ghana records over 24,000 new cancer cases annually, with breast, cervical, prostate, liver, and colorectal cancers being the most prevalent. Sadly, many of these cases are diagnosed at advanced stages when treatment options are limited, costly, and less effective.

In Ablekuma North, as in many other constituencies, we are witnessing an increasing number of families grappling with the emotional and financial burden of cancer. Mr Speaker, the reality is that cancer does not only threaten lives; it impoverishes households, disrupts livelihoods, and places enormous pressure on our already stretched health system.

Hon Members, one of the most worrying aspects of cancer burden in Ghana is late detection. Many of our citizens still lack access to regular screening services, and where such services exist, affordability remains a major barrier. Cultural beliefs, religious beliefs, fear, misinformation, and stigma further discourage early health-seeking behaviour.

Mr Speaker, despite these challenges, Ghana has made commendable strides in recent years. The Ministry of Health and the Ghana Health Service (GHS) have expanded cancer screening programmes, particularly for breast and cervical cancer, at regional and selected district hospitals. The establishment of oncology centres at the Korle Bu Teaching Hospital, Komfo Anokye Teaching Hospital, Tamale Teaching Hospital, and other facilities have improved specialist care. Additionally, public education campaigns through the media and community outreach have increased awareness about cancer prevention and early detection.

However, Mr Speaker, awareness alone is not enough. The greatest obstacle remains financing cancer care. Treatment costs for chemotherapy, radiotherapy, surgery, and supportive care are far beyond the means of the average Ghanaian. The National Health Insurance Scheme (NHIS), though helpful, does not comprehensively cover many cancer-related services and medicines. Consequently, cancer has become in many cases a death sentence for the poor.

It is in this context that I wish to commend His Excellency, President John Dramani Mahama, and the Government of Ghana, for the establishment of the Ghana Medical Trust Fund, popularly known as the MahamaCares. Mr Speaker, this initiative represents a bold and compassionate policy intervention aimed at providing financial support for Ghanaians suffering from chronic and

life-threatening diseases, particularly cancer. MahamaCares offers hope to thousands of vulnerable citizens who would otherwise be denied access to life-saving treatment due to financial constraints. It embodies the principle that access to quality health care is not a privilege for the wealthy, but a fundamental right of every Ghanaian. Hon Members, as a House, we must fully support this fund through adequate resourcing, strong oversight, and transparent management to ensure that it reaches those who need it most.

Mr Speaker, beyond treatment, we must invest more in prevention and early detection. Evidence shows that up to 40 per cent of cancers can be prevented through lifestyle changes, such as healthy diets, physical activity, reduced alcohol consumption, avoidance of tobacco, and vaccination against infections like Hepatitis B and Human Papillomavirus (HPV). We must therefore strengthen health education in our schools, workplaces, churches, mosques, and communities.

Mr Speaker, we must also decentralise cancer services. It is unacceptable that patients from regions like Upper West, Upper East, and parts of Greater Accra travel long distances to access radiotherapy and specialised oncology care. This creates delays, increases costs, and discourages treatment adherence. Ghana needs more regional cancer centres, trained oncologists, nurses, and diagnostic facilities.

Mr Speaker, on this World Cancer Day, I call on this August House to treat

cancer as a national development issue, not just a health problem. A healthy population is the foundation of productivity, economic growth, and social stability. Investing in cancer care is therefore an investment in Ghana's future.

In conclusion, let us commit ourselves to a Ghana where no citizen dies simply because they are poor, where early screening is accessible in every district, where cancer patients are supported with dignity, and where initiatives like MahamaCares are strengthened to become a lasting pillar of our health system. Mr Speaker, the fight against cancer requires unity from Government, Parliament, civil society, the private sector, health professionals, the media, and citizens alike. Together, let us ensure that the message of World Cancer Day, 2026 is not just symbolic, but transformative for the people of Ghana.

I thank you, Mr Speaker.

The Speaker: Thank you, Hon Member. It is now open for comments from Members. Let me start with Prof Beyuo. I am told you are the Chairperson of the Korle Bu Board, which is the reason you are having the first bite.

Prof Titus K. Beyuo: Thank you very much, Mr Speaker, for the opportunity to contribute to this Statement. Let me thank the Hon Member of Parliament for Ablekuma North for making such an important Statement, and for bringing to the fore of

this House the burden of cancers and the challenges that patients and families struggle with daily in this country.

Mr Speaker, cancer is largely a non-communicable disease, even though a few cancers have infectious precursors, like cervical cancer, which is more than 98 per cent caused by the human papillomavirus and therefore can be prevented by vaccination. Similarly, cancer of the liver is largely caused by hepatitis B and C, for which lifestyle changes can lead to prevention or a significant reduction in the burden. And so, in the management of cancer, early detection is key.

I want to support the Hon Member's call that we create awareness and let people screen early, and make the screening services accessible. It is instructive to note, Mr Speaker, that while some cancer care is actually covered by the health insurance, the screening that would actually reduce the cost of people when they eventually have cancer, a lot of the time, is not covered. So, we do not invest in screening, yet when one is diagnosed, the cost of care, which is more expensive than screening, is covered. Screening is key to the prevention. I would also want to commend the Government for one bold intervention that is set to reduce the burden of cancers in the next four to five years. That is the vaccination of young ladies with the human papillomavirus vaccine. Cervical cancer is one of the leading cases of cancers, and with a nationwide rollout of this vaccination last year, it is set to reverse the trend of the prevalence of cervical cancer.

2:55 p.m.

Mr Speaker, as you mentioned, today the Korle-Bu Teaching Hospital received a donation from the Ghana Medical Trust Fund, commonly called MahamaCares, of about GHC298,000 to provide care for four patients with cancers. These are patients with cancer of the cervix, cancer of the brain, cancer of the blood system, what we call a lymphoma, and another patient with tumour of the brain. For these patients and their families, this is life-changing; they are not just another statistic. They are families that otherwise would have been condemned to death. The diagnosis would have meant that they have no chance of survival. But now there is hope for them. To date, the Ghana Medical Trust Fund has supported Korle-Bu Teaching Hospital with close to a million Ghana cedis to care for both cancers and heart conditions. Today, they visited a patient who had a deep-seated brain tumour that was successfully treated because of Ghana Medical Trust Fund. So, we want to commend the President for such a huge intervention.

I would want to say that there is hope for patients living with cancer because of this intervention. But do not wait to be diagnosed late because if the diagnosis comes late, no matter how hard medical experts try, we are not able to control it. So early detection remains key, and that is where the window of intervention is still possible.

So, I commend the Hon Member once again for this Statement and hope that this House would help the

Government in passing policies and laws that will ensure equitable access to screening services for cancers and treatment across the country. It should not be possible that based on where you are located, diagnosis of a particular cancer to you means a death sentence. That is not acceptable.

Thank you very much, Mr Speaker.
[Hear! Hear!]

The Speaker: The Hon Ranking Member of the Committee on Health was on his feet. Am I right, Doctor?

Yes, Hon Ayew?

Dr Nana Ayew Afriye (NPP — Effiduase/Asokore): Mr Speaker, I rise to associate myself with the maker of this Statement. I want to say that the theme for Ghana's celebration is "Community Action and Closing the Gap". I look forward to seeing the Ghana Medical Trust Fund allocating resources properly. And when they do so, it is going to be in enhancing specialist care and setting up of specialist centres. This is the only good thing, as I said on the Committee, that stands to benefit us, apart from the triplication and duplication of the Trust. If indeed the Trust stands for what it wants to achieve, Hon Colleagues, in this country, where it finances entirely specialist services and establishing of subspecialty centres, then indeed it is the only good thing that can be associated with this Ghana Medical Trust Fund. We stand to see if indeed that is the objective. And very well, there will be hope for Ghanaians if they stick to and are very disciplined and they stay

focused on achieving centres with specialty and centres with subspecialty care.

Mr Speaker, cancer as it is, is a bit of a family predisposition and a bit of a lifestyle to say. For family predisposition, well, it is unfortunate, but we look at the behaviour where Ghanaians would encourage themselves for frequent self-examination and for screening services. Indeed, the National Health Insurance Authority's package in 2024 approved of health promotion and prevention. So, if they are going to roll it from 2025 forward, then we should be having answers when they come with their formula. Mr Speaker, they are coming with a formula, probably this month, and your Committee would ask them that when they included promotion and prevention in the benefit package, what has come out of it? We will report to you entirely what Ghanaians want to hear because your Committee stands by you and they will do the right thing to assure Ghanaians that whatever programme they have, the feedback would show it is going well.

Mr Speaker, my association with this Statement is just to say that the people of this country, and we in Parliament are aware of the fact that apart from family predisposition, lifestyle choices such as drinking alcohol and smoking are also factors. And Mr Speaker, we are looking forward to a point where you may launch an initiative that all will be ambassadors just to drum the issue home for Ghanaians to know that two things that can reduce the burden of cancer are avoiding drinking and smoking, and

also, eating of healthy foods such as fruits, and exercising to reduce needless weight gain. This is something that we can all associate ourselves with and I am sure that when you charge us to the core, the good people of this country will know that we are here not to only formulate policies or look at allocation of resources, but when it comes to prevention and promotion, we in Parliament will stand by any initiative to create that awareness.

I thank you, Mr Speaker, for this opportunity to speak on this Statement.

The Speaker: Yes, Hon Laadi Ayii?

Hajia Laadi Ayii Ayamba (NDC — Pusiga): Thank you very much, Mr Speaker, for the opportunity to contribute to the Statement ably made by my Colleague on this occasion.

Mr Speaker, since my coming into Parliament in 2013, year in, year out, we continuously make Statements, we contribute and we say all the good things about cancers, the various types of cancers, we discuss them. Unfortunately, Mr Speaker, one of the greatest problems that we have is walking the talk. The walking is so minimal as compared to the talking.

Mr Speaker, prevention is better than cure, is an old adage which is important, yet, our preventive methods are minimal and very slow, because it goes with a lot of education. It is important that education is given not only in communities, for instance, where health personnel visit, but even in the schools.

We have just had these preventive vaccines that have been started. How far has this gone? How many schools have been visited? How many girls have been vaccinated? Is it continuing? How fast is it going? Because as and when the disease comes, nobody knows. Yet, we continue staggering. It is very important for us to do it continuously and vigorously, and give it all the seriousness that it deserves while we ensure that decentralisation is taking it up very fast.

We have just heard about moneys that have been released for the MahamaCares, and I believe that this is a national anthem that we on this Side had when we were going round, campaigning.

3:05 p.m.

I believe once the money has come, Mr Speaker, disbursement to the various districts is key. We should not keep the money in Accra and wait for someone in Pusiga to travel to Tamale to go and do screening.

Mr Speaker, as mentioned earlier by my Hon Colleague, screening is very important. But where will the women in the villages get the money to do the screening? Before we even talk about treatment, we need to ensure that screening is undertaken in all our districts, and if possible, the Community-based Health Planning Service (CHPS) compounds in the various villages and data should be properly acquired to ensure that whatever treatment or support is being given, the data will inform where and what.

Mr Speaker, as we continue making these statements, if we do not act and ensure that the action is immediate and to the needed places, everything will continue to be in either Accra or just the regional capitals or the regional hospitals, and the woman who we are worried about and talking, especially when it comes to cervical cancer, would not even know about it. So, I will plead with all of us in this Chamber to ensure that the message continues to go round that prevention is better than cure, and it is better that every woman gets tested or scanned, and that the work with the schools is done quickly.

Thank you, Mr Speaker, for the opportunity to contribute.

The Speaker: Yes, is that Hon Zuwera?

Ms Zuwera Mohammed Ibrahimah: Yes, Mr Speaker, your one and only daughter.

The Speaker: Ei, I have more daughters. You are not the only one.

Ms Zuwera Mohammed Ibrahimah (NDC — Salaga South): *[Laughter]* Thank you very much, Mr Speaker, for the opportunity to contribute to the Statement made by my Hon Colleague, the Hon Member of Parliament for Ablekuma North.

Mr Speaker, I could not agree more with the Hon Member for the thought and depth of the Statement that she has presented to this House today. The theme for this year's World Cancer Day is

United by Unique. A look and deep interpretation of the theme tells us that cancer and what it presents manifests differently in different individuals and different personalities. Therefore, there is no one standard care for any case of cancer. What this means is that our health institutions, our health professionals must be adequately armed and skilled to provide customised cancer treatment to different people who have cancer and which cancer manifests differently.

Mr Speaker, growing up, we heard about cancer, but oftentimes we heard about breast cancer, and we heard about cervical cancer. We did not hear a lot about prostate cancer or Leukaemia. We did not hear much about the different types of hepatitis, A, B, and C. Suddenly, these cancers have become an everyday thing for everybody. Cancer today has become like malaria and fever as we knew them. And in that direction, Mr Speaker, we will want to commend and laud the initiative of His Excellency the President by establishing the MahamaCares, which goes outside the National Health Insurance Authority (NHIA) prescriptions, to provide support and assistance to people who have cancers, diabetes, and other such diseases.

Mr Speaker, in this country, it is very hard to confidently speak about cancer, whereas in other jurisdictions, the post box, as we call it, the letterbox, is a lifesaver for people. I say this because, if one goes to the United Kingdom, for example, the health directorate holds all the data on its patients; it knows when

one is due for their next cancer screening. After 40, the health providers make it their business to remind them, even if they have forgotten, that they are due for their next screening at a particular time. That letter will arrive in their letterbox with the famous NHS inscription.

Mr Speaker, we have heard Prof Titus Beyuo tell us that, inasmuch as the MahamaCares, that is the Ghana Medical Trust Fund, will support the treatment of cancer patients, we do not have the facilities to offer screening to enable detection. This Statement tells us that early detection actually leads to better treatment. Mr Speaker, it is time that we begin to look at the end of dealing with the cancer issue. We need to invest in acquiring the data during the screening process. We need to invest in systems and processes that will ensure that all of us get screened when we have to be screened in order to curtail the devastating effects of cancer.

Mr Speaker, there are three food items in this country. I am not a scientist, I do not know whether my Hon Colleague scientists here will help, but I have identified, for example, tomato, which is a constant ingredient that we use in a lot of the meals that we prepare at home. These days, when one buys tomatoes at the market, they are ripe on the outside and green inside when cut open. We do not know what is causing this. We do not know what effect it has on our health when consuming tomato that is ripe on the outside and green on the inside. These are the tomatoes we have in our markets today.

Mangoes, when we buy them, look ripe and beautiful in the store. When we take it home, wash and cut, the mango is ripe on the inside and rotten, but the outside is still unripe. Mr Speaker, we all grew up plucking mangoes around our neighbourhoods and eating them. We never experienced this. We do not know what is happening. The mango is so sour that one cannot bite it.

Mr Speaker, another fruit that has become difficult to eat in this country is pineapple.

3:15 p.m.

The pineapples we eat in this country are so sour that we do not know what is causing it. We do not know what our regulatory bodies are doing, whether it is the Food and Drugs Authority (FDA) or the Ghana Standard Board; we do not know. All we see is food on the market, and we are purchasing it to eat.

Mr Speaker, we do not know what the direct correlation is between what we eat and the sudden proliferation of cancer in our system. I think we need to take a step back and look properly at what we eat. Our authorities must be up and doing and protect all of us from unwholesome food. Our tomatoes are not good. Growing up, our parents tossed tomatoes for us in the morning as breakfast. Today, if we toss it for breakfast, nobody will eat it because it is sour. We eat mangoes and pineapples when we travel abroad because that is when we get the best. In our own country, the mango or pineapple that is produced, we cannot bite into it.

So, Mr Speaker, in adding my voice to the Statement, I will urge our authorities, since treatment is the way to go once the disease strikes, we should also focus on prevention, and there is a lot that we can do to prevent the cancer that we are all struggling with.

Thank you very much, Mr Speaker, and thank you, my Colleague, Ms Ewurabena, for this brilliant Statement.

The Speaker: Yes, Prof?

Prof Kingsley Nyarko (NPP — Kwadaso): Mr Speaker, I am grateful for the opportunity to make a few comments on this important Statement, and I want to also welcome you back to the House.

Mr Speaker, this is a very emotive subject, and I have a personal experience with that. Any time cancer is mentioned, I find myself in a very difficult situation.

Mr Speaker, I remember in the year 2013, I lost my second child to leukaemia. She was born in Munich, Germany, and I was wondering what might have caused her to have cancer. So, I decided to relocate to Canada because I used to have a stay permit. I travelled to Canada, and the next morning, I was watching Television, and I saw that leukaemia was the number one killer of children. Then I said to myself that I ran away from this particular disease, but that disease is prevalent in this country. So, why should I live there? So, I decided to come back to my country.

Mr Speaker, I have some very close family members who have died from cancer, and now we are talking about MahamaCares. As much as I would not want to say anything untoward about MahamaCares, for me, the issue has to do with how we find a lasting solution to this particular disease because the devastation is huge. The effects on families in terms of their psyche, well-being, and their resources. This is what we should be looking for: our environment. Is it the issue about our environment? What is actually causing this disease?

Mr Speaker, when cancer hits someone, unless divine intervention, I do not know how the person can come out of cancerous conditions. So, we need to find a workable solution beyond treatment, and beyond setting up some of these programmes. I heard a previous speaker, Prof Beyuo, talking about GHC1 million in the MahamaCares. The amount of GHC1 million is a paltry figure. My point is that if we want to deal with this and deal with it well, so that those of us who have had disastrous experiences with this canker could be comforted, then we need something that goes beyond GHC1 million. We need to get others to come on board. We need the private sector, entrepreneurs, and other persons to help.

Mr Speaker, the cost burden cannot be borne by an individual alone. So, it is an important topic. All of us are at risk; our families are at risk, and especially our children. So, let us do more with this. It should not be about praising MahamaCares, it should be more about

finding some resources and creating some form of huge fund. Not GH¢1 million, but billions of cedis because we need to do serious and extensive research to be able to get to the bottom of it, and I am sure one day we will get there. If we put rhetoric aside and we become more intentional, I am sure that we can give a deadly blow to cancer.

Mr Speaker, when one goes to the hospital to see a doctor, and the labs show that the person has cancer, I am sure the person would start writing or preparing their will because the possibility of survival is minimal unless God intervenes —

The Speaker: Prof, I am sure your reference was just general and not to me.

Were you talking to me?

Prof Nyarko: No, Mr Speaker.

The Speaker: Yes, make it general because when you say, “Mr Speaker, if you...”

I do not write my will like that. I do not fear death; not me.

Prof Nyarko: Mr Speaker, God willing, you will live beyond 100 years.

The Speaker: Oh, no, only 100 years.

Prof Nyarko: Your expertise is needed by us and the next generations. You are not going now by the grace of God.

Mr Speaker, I thought you would say amen. *[Laughter]*

The reason I am deliberate on this is that three members of my nuclear family: my daughter, her sister, and their mother, all died from cancer – not a particular cancer. So, it hits me, and I am thinking—I remember at the Legon Hospital, the doctor was bold to tell me that if we even took her to India, she would not survive. But can we do something extraordinary so that others would not suffer the suffering some of us went through?

So, to the maker of the Statement, Member of Parliament (MP) for Ablekuma North, we thank you for this. But I hope that next time we make this Statement, at least more progress would have been made, more moneys have been found, not only for the MahamaCares, but also for other donors supporting us to have a pool of resources to help us deal with this once and for all.

Mr Speaker, thank you, and may you live long.

The Speaker: Hon Members, I know this is a very important subject matter. We even agreed last year to contribute to support the Breast Society of Ghana, because of this issue of breast cancer.

So, Hon Leaders, do we move on, or do we take more?

Mr Shaib: Mr Speaker, I would have loved to contribute, subject to some sort of agreement with my Hon Majority Leader. Maybe, we can take one from

each Side and then one-one from Leadership because we can see there are a lot of —

03:25 p.m.

The Speaker: So, I take one from each Side, and Leadership, which means two.

Mr Shaib: That is so, Mr Speaker.

The Speaker: If that is the case— Yes, Dr Amin?

Dr Mohammed Amin Adam (NPP — Karaga): Mr Speaker, I am your son, actually.

The Speaker: Yes, some years ago. *[Laughter]* These days, I do not know whether you are still my son.

Dr Adam: Mr Speaker, prodigal son. *[Laughter]*

The Speaker: You got a new father, so I do not know.

Dr Adam: Thank you, Mr Speaker, for the opportunity to contribute to this Statement. It is a very important Statement which must strike the attention of this country. Particularly, I am interested in the economic impact of cancers, and the impact is quite huge: productivity loss and the direct cost to individuals who will have to spend on screening, treatment and transportation costs. But also important, Mr Speaker, is the cost to the nation as a result of early mortality from cancers. If we give attention to cancers, we will help in

revitalising our economy from the shocks associated with cancers. I do appreciate the support provided by the National Health Trust to Korle-Bu for the treatment of cancers. But, GH¢1million is nowhere near what is substantially required to address the issue of cancers. We need to commit more resources to the issue of cancers.

Globally, it is estimated that the economic impact of cancers between 2020 to 2050 amounts to US\$25 trillion. I am not sure we have data on the economic impact on Ghana yet, but it is important that we gather that data. This is important because if we do not have that data, we will not be able to plan how to address issues related to cancers. And so, Mr Speaker, I want to make a call to the Government to give it more attention, to get State institutions to get us the data. Data is sketchy, and we need that data to be able to plan on resource allocation towards addressing cancers.

Mr Speaker, I do recall that since 2011, Ghana has implemented the National Strategy for Cancer Control, which was updated between 2021 and 2025. Therefore, the implementation of the strategy would have ended last year. We do not know if there is an updated plan from 2026 onwards. Therefore, Mr Speaker, I would want to recommend that we call on the Hon Minister for Health to brief this House on the implementation of that national strategy for the control of cancers and also to inform us what the next plans are. If we are jettisoning the plan which is supposed to have concluded by last year, do we have a new one that is replacing

that? If we do not have a new one, when are we going to have a new one so that we can get the direction on how we address issues relating to cancers.

Mr Speaker, this is important because I hear many people talk about screening. What resources are we making available to support screening? If we are to leave it to individuals, we will have serious problems. Issues relating to promotion; this is an issue the Government should take on upon itself. Private individuals, private organisations, and non-governmental organisations (NGOs) can do their best, but Government needs to rally the resources, marshal all the energy at our disposal to support the promotional side of this cancer. Promotion is so important because if we do not create awareness, people may be carrying cancers without even knowing. This is why screening, early detection, and treatment are important and that we cannot avoid any of this within the treatment of cancer. Mr Speaker, this is my call, that the Minister for Health comes to this House to brief us on the implementation of the national strategy and when the next strategy will kick off. I thank you, Mr Speaker, for the opportunity.

Some Members — *rose* —

The Speaker: Yes? [*Pause*] Well, I will give it to the Doctor. Yes, even though I have bias for Hon Ladies, this one, I think the Doctor has won the day. Yes, please, Dr Anabah?

Dr Thomas Winsum Anabah (NDC — Garu): Thank you, Mr Speaker, and thank you, Hon Colleague,

for ceding the opportunity for me. I know the Speaker “likes” women to be given more privileges—

The Speaker: Please, what word did you use? [*Laughter*]

Dr Anabah: Mr Speaker, I said, “prefer”.

Mr Speaker, thank you very much for the opportunity. I rise to contribute to the Statement made by my Hon Colleague regarding the Day of Cancer on the theme “United by Unique”. I think a lot has been said, but some few observations are needed to be brought to light regarding screening and prevention. In medicine, it is said that much of our problems can be solved through prevention and that primary health care can take care of 70 per cent of our health needs and reduce morbidity and mortalities. If we actually listen to what Hon Colleagues have said here and the little experience I have had with my mother when she developed cancer in 2020 and the struggle I had to go through to get her from Tamale to Kumasi to get treatment provided to her, I tend to sympathise with anybody who has a family member with cancer who is from a deprived area, a deprived community, and a deprived family. That is why I think that Government needs to invest more in primary health care where prevention and early diagnosis can be done, and treatment provided very early to minimise the cost and the burden on families. For example, I have evidence to that in the entire northern sector, the five regions, we only have one mammography for breast cancer. That is very bad. It

means that if a person does not go to the Tamale Teaching Hospital from the five northern regions, they cannot be diagnosed of breast cancer through a mammography, this is very poor. This means that a lot of women who have developed breast cancers may even end up at an advanced stage before they are sent to Kumasi or to Accra for treatment. My call is that we invest very well in primary health care and also in making early diagnosis and, where possible, give vaccinations for prevention.

How much is Prostate-Specific Antigen (PSA) to diagnose a prostate cancer or the possibility for somebody to develop prostate cancer? It is peanut; it is not up to GH¢200. But we still see men developing what we call prostate cancer even though it can be diagnosed very early, and treatment sometimes with just some tablets or, in certain cases, the removal of the test is given at that stage could prevent serious illness so that no prostate cancer develops and the person experiences no further problems. How much is a pap smear screening that we cannot provide in all our village hospitals or even every district hospital, to diagnose cervical cancers in women? So, primary health care is something we have to look into it very seriously.

Hepatitis, how much does it cost to test someone for Hepatitis B and C? Can it not be part of the national programme that every year, every person in Ghana is tested for Hepatitis B and C and vaccinated? Then those who are vaccinated would not need to be tested again the following year. This would also help reduce the prevalence of

cancer, particularly liver cancer, in the population. But as the saying goes, the journey of a thousand miles begins with a single step.

3:35 p.m.

At least the MahamaCares has come in to start doing something. I would like to appreciate His Excellency, the President for this bold initiative to start something to treat people with chronic non-communicable diseases and people with high-burden diseases that require a lot of financial resources. But I will call on Government to invest more in primary health care. The CHPS compounds we are developing now, two CHPS compounds for every district every year for the next three years, will give us thousands of CHPS compounds where pap smear tests can easily be done, and where Prostate-Specific Antigen (PSA) can easily be tested on a rapid diagnostic kit to know whether somebody will develop prostate cancer or cervical cancer. Also, every district hospital can be given a mammography machine to make sure we scan all our women every three years to see whether they are developing something in the breast.

For me, we should hail the MahamaCares. We should not condemn it but we should rather ask them to innovate it. To buy more resources to the lower hospitals to do early diagnosis, to minimise the cost of treatment and the economic burden on families that need to treat their family members with advanced stages of cancer. To this end I say Mr Speaker thank you for permitting me to contribute a little to this Statement.

Mr Jerry Ahmed Shaib (NPP — Weija/Gbawe): Thank you very much, Mr Speaker for this opportunity. I stand to contribute to the Statement made by the MP for Ablekuma North.

Mr Speaker, I have averted my mind to a few issues. One of which is that even with this Statement we have had some political twists to it. Instead of people concentrating on the gravamen of the issues, we would realise that, respectfully, almost every speaker on the Majority Side found reason to link it to the Ghana Medical Trust Fund and went on propagating the MahamaCares. But what we have also fallen short of is to remind ourselves that the Ghana Medical Trust Fund is taking its source of funding from the National Health Insurance Fund, which was initiated by the former President John Agyekum Kufuor.

To continue, we would also understand that when we go to the Korle-Bu Teaching Hospital, one of the biggest health facilities that is used in treating cancer is a urology and nephrology centre which was constructed and commissioned by former President Nana Addo Dankwa Akufo-Addo. When we come to Ghana there are major issues we have to deal with when it has to do with cancer. The statistics are startling.

First of all, when we look at the Global Cancer Observatory (GLOBOCAN) report of 2022, it tells us that annually, new cancer cases in Ghana are 28,000. That is something to worry about instead of doing the usual politics. It also tells us that, annually, almost 18,000 people die of cancer in the

Republic of Ghana, therefore we need to be concentrating our efforts on prevention, awareness and education.

This is why as a Member of Parliament one should also not take this for granted. If the person is a man, he should ask himself the last time he tested for the PSA? Has he ever had the opportunity of testing the PSA? They are women, have they had any breast screening? If they have not done that, do they take diet seriously? How are they eating and exercising? Not just rushing to come and sit in the Chamber. These are all very critical issues.

Mr Speaker, this is another important issue we have not looked at. With the surge in *galamsey* there is a lot of heavy metals in food. That is why I would corroborate Hon Zuwera's submission on the kind of food and fruits we are having. When we take mangoes, pineapple or tomatoes which we have been told can even help in curing cancer, what is happening to them? Just because of the surge in *galamsey* people are having all manner of issues. These are some of the things we have to avert our minds to, to see how best we can, if not eliminate *galamsey*, to curb *galamsey*.

This World Cancer Day is supposed to pay attention to three major group of people. One, are the fighters of cancer; those who have survived. How have they gone by that? How have they survived? What did they do? The caregivers, when we go to Korle-Bu Teaching Hospital, they do not have a lot of the resources or facilities. But when one goes to see Professor Mensah and his team, they are

doing extremely well in providing service to ensure that people's lives are saved. We need to commend these people.

Finally, we need to also pay homage to the people who have lost their lives and express our sympathies and condolences to them. I heard Prof Kingsley Nyarko talk about losing his daughter through leukaemia and that is a very sad story. Our prayer is that we should all depart from the usual politics and come together to ensure that we put a forceful mind to solving this problem, this canker called cancer, so that we can have a better life. World Cancer Day reminds us that cancer is not just a medical battle but a human one. Together we can create awareness, together we can prevent cancer and together we can save mother Ghana. Thank you very much, Mr Speaker for this platform.

The Speaker: The last person I am told by the—Hon Deputy Majority Leader, will you do it on his behalf?

Mr George Kweku Ricketts-Hagan: No, Mr Speaker, I was going to ask for your permission to let the Hon Member for Nanton take the slot of Leadership.

Dr Abdul-Khaliq Mohammed Sherif (NDC — Nanton): Thank you very much, Mr Speaker for the opportunity to contribute to this Statement made by the Hon Member for Ablekuma North.

First and foremost, Mr Speaker, I would want to say a very big thank you to all health care workers in this country, the doctors, nurses and midwives, for the great work they are doing for Mother Ghana. Cancer, as we all know, is one debilitating disease that one would never want to encounter. I recall my few days as a medical officer in Salaga, where I came across a 35-year-old who inadvertently had a mass removed from her breast, and the person who removed this mass did not know that they did not have to throw it away. The mass was thrown away and she came to me after five years with full-blown breast cancer.

But thankfully, Mr Speaker, we now have a Government in place. In this Chamber, we have had a lot of things being said about screening, and that screening is the way to go. Screening is what we have to do to catch the disease early so that we can treat it well. Thankfully, the Ministry of Health, together with H. E. John Dramani Mahama, are coming up with this policy of free primary health care. The essence of free primary health care is to bring screening and early detection to the doorstep of the ordinary Ghanaian.

What it means is that with this policy, any woman, any child or anyone can walk to the nearest CHPS compound and have their breast examined. Any woman can walk to the nearest health centre and have her cervix examined. Anybody can walk to the nearest CHPS compound and have their glucose checked. Anybody can walk to the nearest health centre and have their blood Pressure (BP) checked; that is what we mean by screening.

3:45 p.m.

Mr Speaker, the government is not only stopping at that but says that once a person screens and they find an at-risk population, what do we do? We would refer to the next available facility. Then the National Health Insurance is going to take its way. Issues that cannot be solved by the National Health Insurance, like the childhood and breast cancers and others that we are talking about, then we would have the Ghana Medical Trust Fund come in.

Mr Speaker, I would want to put it on record that it is not only the provision of payment of medical bills that the Ghana Medical Trust Fund is looking at. The Ghana Medical Trust Fund is looking into the training of doctors.

I can tell you for a fact that this year, the Ghana Medical Trust Fund, in partnership with the Ghana College of Physicians and Surgeons, is looking forward to training doctors and specialists for the Upper West Region, Northern Region, Bono Region and so on. There is no way one can fight cancer if one does not have an oncologist and an oncology nurse.

The same Ghana Medical Trust Fund is looking forward into doing research and innovation. We are setting up what they call the cancer registry. Sometimes, cancers come from certain locations and towns. If we have a cancer registry, then we can tell that a certain type of cancer is most likely coming from Nanton and another type is likely coming from Tano.

If that is the case, can we go back and see what the risk factors in these areas are?

Mr Speaker, this is the holistic approach that this government is taking. What is the whole reason? It is for us to achieve what we call universal health coverage, which says that nobody, no Ghanaian, no individual should be denied of health care, irrespective of distance, religious or tribal affiliation.

Mr Speaker, before I take my seat, there is this impression that has been created that, the Korle Bu Teaching Hospital got a mere GHC1 million and just GHC248,000 was paid. Today, four patients who otherwise had no hope and would have been on television telling the whole world to pay to a certain mobile money account number—Four of them have benefited; those four families are thankful that their worries and woes have been taken over by the Ghana Medical Trust Fund. *[Hear! Hear!]*

Again, the Ghana Medical Trust Fund is retooling our health facilities. In the fight against cancer, diagnostics are very essential. If we do not have mammography machines, how do people get screened for breast cancer? If we do not have an MRI or a PEP Scan, how are we going to manage our cancer patients? That is what Ghana Medical Trust Fund is doing. That is why we say that with the Ghana Medical Trust Fund, truly, Mahama Cares.

Mr Speaker, thank you very much for the opportunity. *[Hear! Hear!]*

The Speaker: Hon Members, I think it is time for us to take the second Statement. The second Statement stands in the name of Dr Kingsley Agyemang, the Member of Parliament for Abuakwa South. It is on the 61st Anniversary of the passing of Dr Joseph Kwame Kyeretwie Boakye Danquah. Please, Hon Member, you may do so now.

**A Commemorative Statement
Marking the 61st Anniversary of the
Passing of Dr Joseph Kwame
Kyeretwie Boakye Danquah**

Dr Kingsley Agyemang (NPP — Abuakwa South): Thank you very much, Mr Speaker.

I rise, pursuant to Order 92 of the Standing Orders of the Parliament of Ghana, which permits a ceremonial speech confined to the commemoration of a special event and the eulogising of a renowned and distinguished deceased person to mark the 61st anniversary of the passing of Dr Joseph Kwame Kyeretwie Boakye Danquah one of the foremost intellectual architects of our nation.

On 4th February 1965, Ghana lost a philosopher, jurist, nationalist and a moral voice whose ideas continue to inform our constitutional and democratic life. In recalling his life, one is reminded of the words of scripture in Proverbs 4, verse 7, and I quote, “Wisdom is the principal thing, therefore get wisdom, and in all thy getting, get understanding.”

Dr Danquah’s life was a living embodiment of this injunction. He sought wisdom not for personal elevation, but for national awakening.

Mr Speaker, over the years, many worthy and illuminating tributes have been laid in honour of Dr Danquah. Scholars have examined the richness of his intellectual life. Jurists have reflected on his contribution to constitutional thought. Historians have chronicled his role in the nationalist movement. Political leaders within his own tradition have rightly celebrated his courage, sacrifice, and vision. Taken together, these attributes have presented Dr Danquah as a many-sided figure whose life defies narrow classification.

The purpose of this Statement is therefore not to repeat what has already been eloquently said, nor merely to rehash the familiar catalogue of his achievements. Rather, the distinct thrust of my intervention this afternoon is to focus attention on a more exacting and I submit, more compelling measure of Dr Danquah national stature. The manner in which he has been eulogised, acknowledged and intellectually engaged by those who did not share his political ideology and in some instances, stood in firm opposition to it.

Mr Speaker, it is on this basis that this Statement proceeds to examine selective reflections and assessments of Dr Danquah by persons and traditions that do not align with his political worldview. In doing so, I seek to establish beyond sentiment and party that Dr Danquah stands as an undisputed national figure whose legacy belongs to the entire Republic and not to any single political inheritance. It is in this spirit that this House is invited to reflect.

Mr Speaker, few testimonies concerning parliamentary excellence carry greater institutional authority than those of a Clerk to Parliament. In this regard, the reflections of Mr K. B. Ayensu, Clerk of the National Assembly from 1955 to 1966, remain compelling. Writing in echoes from the past, timeless tributes to J.B. Danquah in 1967, Mr Ayensu observed that,

“After Dr Danquah had left the parliamentary scene, slowly but inevitably, the rot set in. The proceedings began to acquire a certain element of insipid farce. Many in the House privately wished for a return to the Dr Danquah days.”

Mr Speaker, professional institutions whose legitimacy resets on neutrality have equally affirmed his legacy. In 2025, the Ghana Bar Association publicly reiterated that,

“Dr J. B. Danquah’s role in Ghana’s struggle for independence remains unparalleled.”

The association recalled his leadership in the Aborigines’ Rights Protection Society, which successfully resisted colonial attempts to expropriate Ghanaian lands and noted that this early activism marked the beginning of a sustained legal struggle against colonial injustice. Similarly, the Ghana Academy of Arts and Sciences, an institution entirely removed from partisan contestation, had documented Dr Danquah’s decisive role in national formation.

The Academy recalls his instrumental contribution to the Burns Constitution of 1946, his foundation membership of the United Gold Coast Convention in 1947 and his description by the Watson Commission as the “Doyen of Gold Coast Politics.” Mr Speaker, contemporary policy analysis has reinforced this historical assessment.

In 2025, African Leadership and Policy for Humanitarian Action (ALPHA), a pan-African think tank, stated that the history of Ghana and Africa would be incomplete without recognising Dr Danquah’s contribution to the constitutional development and democratic consolidation.

3:55 p.m.

Mr Speaker, perhaps most persuasive for this House are the voices captured in our own parliamentary *Hansard*, spoken by Hon Members from across the political divide. On 5th February, 2022, Hon Rockson-Nelson Etse Kwami Dafeamekpor, now Majority Chief Whip, stated on the floor of this House that although Dr Danquah lost elections and came into conflict with the security apparatus of the state, and I quote; “his contributions and criticism of the government”, were significant, and that he was “a towering figure of legal mind.” On that same day, Mr Speaker, on that basis, at his instance, this House observed a minute’s silence in honour of Dr J. B. Danquah, a solemn act that speaks profoundly to bipartisan respect.

On Tuesday, 4th February, 2025, Hon Mahama Ayariga, Majority Leader, in

contributing to a Statement moved by my good self, reminded the House, and I quote directly from the *Hansard*; “you cannot write the history of this country without recognising the contributions of Dr J. B. Danquah.” He further drew attention to Dr Danquah’s enduring scholarly legacy in Akan customary law, noting the continued relevance in legal education. Earlier on Wednesday, 5th February, 2020, Hon Kwame Govers Agbodza, the then Minority Chief Whip, cautioned this House that the experience of Dr Danquah under the Preventive Detention Act should permanently remind legislators to consider the long-term consequences of laws passed in moments of political urgency.

On Friday, 5th February, 2016, Hon Ahmed Ibrahim, Member for Banda, highlighted Dr Danquah's central role in the Africanisation of the Legislative and Executive Councils and in securing the establishment of the University of Ghana, observing that this single contribution had produced generations of intellectuals who now serve the Republic.

On Thursday, 5th February, 2015, Hon Haruna Iddrisu, Member for Tamale South, acknowledged Dr Danquah’s advocacy for farmers, his role in the establishment of the Cocoa Marketing Board, and his brilliance in legislative debate, concluding that his contribution to Ghana’s independence was “enormous and undeniable.”

On that same date, Hon Fritz Baffour, former Minister of State, delivered what may be one of the most profound

parliamentary affirmations of Dr Danquah’s national stature when he stated; “It was people and men like Dr J. B. Danquah who gave us the ethos of nationhood. He gave us the name of the nation. He gave us the purpose of the nation”.

Mr Speaker, on Wednesday, 5th February, 2014, both Hon Benjamin Kunbuor, former Attorney-General, and the then Hon Alban Sumana Kingsford Bagbin, now Speaker of this House, reflected on the devastating impact of the Preventive Detention Act, using Dr Danquah’s experience to caution against what was described as constitutional dictatorship, and to emphasise that such an ordeal should never be visited upon any citizen, regardless of stature.

Mr Speaker, when clerks of Parliament, professional bodies, academic institutions, policy think tanks, Majority and Minority Leaders of the National Democratic Congress (NDC), Ministers of State of the NDC, and Speakers of Parliament, speaking across decades and ideological divides, converge in the assessment of one man, then history itself has rendered its verdict.

Mr Speaker, history is often most honest when it speaks across divisions. A man whose life and work are acknowledged with respect by those who oppose him politically, whose ideas continue to frame constitutional governance and democratic restraint, and whose personal sacrifice remains a warning and a lesson to successive generations, meets every test of national

heroism. The experience of Dr J. B. Danquah under the Preventive Detention Act reminds this House that the erosion of liberty rarely announces itself loudly, but often returns quietly through practices that normalise excessive restraint, prolonged detention, or punitive bail conditions that test the boundaries of constitutional reasonableness. It is therefore not as a political claim, but as a historical conclusion that this statement submits that Dr Danquah's life, ideas, and sufferings compel us to eternal vigilance. To honour him is not merely to remember his past, but to ensure that the dark chapters he endured are never reopened in our present.

Mr Speaker I am most grateful to you, for the opportunity and the space afforded me to make this Statement. Thank you.

The Speaker: Thank you, Hon Member. It is now for the House to comment on.

Mrs Korkor Laurette Asante (NPP — Atiwa West): Mr Speaker, thank you, for the opportunity to comment on the Statement made by the Hon Member for Abuakwa South.

Mr Speaker, we remember Dr J. B. Danquah, the scholar, the freedom fighter, the jurist, the author, the statesman, and the historian. We remember Dr J. B. Danquah as the leader who defended the freedom of Ghanaians and worked for a liberal, democratic form of government underpinned by the rule of law. We recall the case of *Re:*

Akoto and 7 Others in 1961, where Danquah brought an application for an order for a *habeas corpus* for the release of Baffour Osei Akoto, who was then the senior linguist for the *Asantehene* and was also a founder of a political movement. Although the legal challenge brought by Dr Danquah failed in terms of the arbitrary detention of a citizen, we in Ghana today enjoy the benefits of the powers of the Supreme Court to determine the constitutionality of legislation and to strike down unconstitutional legislation. Today, we are the beneficiaries of the principles and freedoms enshrined in the Constitution.

Mr Speaker, I believe that Dr Danquah's fight as a jurist for the rule of law in a liberal, democratic form of governance has borne fruit in the present-day Ghana that we all enjoy.

Mr Speaker, thank you for allowing me to contribute to the Hon Member's Statement.

Nana Agyei Baffour Awuah (NPP — Manhyia South): Mr Speaker, thank you very much. Being the MP for Manhyia South, where Nana Baffour Akoto was a constituent and one of the founding fathers of our political tradition, it would be unfair to the memory of Dr J.B. Danquah if I do not contribute to the Statement.

Mr Speaker, in fact, it is also on record that the foremost constituent of Manhyia South, Otumfuor Osei Tutu II, has also acknowledged that Dr J. B. Danquah was also the lawyer for Nana Osei Agyeman Prempeh II. So, respectfully,

being their MP, it is only fair that on this day I add my voice to the comments so far made.

Mr Speaker, history should unite us as a country rather than divide us. It is important that as a nation we acknowledge each other's contribution to the development of this country rather than berate it. It does appear that oftentimes in our debates as to who contributed to the independence of this country, we want to berate others and then emphasise the importance of the roles of others.

Mr Speaker, if for nothing, today, Chapter 5 of the 1992 Constitution, on the fundamental human rights of the people of this country, traces its source to the arguments Dr J. B. Danquah made in his *habeas corpus* application for Baffour Akoto, a deceased important member of my Constituency. On that score alone, if for nothing, we cannot overlook Dr J. B. Danquah's contribution.

4:05 p.m.

On this occasion, on behalf of the people of Manhyia South, we acknowledge the importance of the role of Dr J.B. Danquah, and I commend my Brother from Kyebi, which is also a very important traditional political seat, for the Statement made.

Thank you very much, Mr Speaker.
[Hear! Hear!]

Mr Samuel A. Jinapor (NPP — Damongo): Thank you very much, Mr Speaker, for the opportunity to contribute

to this very important Statement commemorating the passing of Dr J.B. Danquah, ably made by my good Hon Brother, the Member of Parliament for Abuakwa South. Mr Speaker, celebrating one of our historic forebears and figures of our country does not in itself amount to diminishing the contribution of other great leaders. It is in this spirit that I believe it is fitting that this House is paying such tribute to the memory and work of Dr J.B. Danquah.

Mr Speaker, Dr J.B. Danquah's legacy cuts across various spheres of our national life. It cuts across politics, it cuts across the development of our national economy, the arts centre, history, academia, intellectualism, and many more. Indeed, Dr J.B. Danquah, as far back as 1946, was one of the key driving figures for the best constitution of our country, which some pundits have argued as the fulcrum on which our independence struggle was fought.

Mr Speaker, in the formation of the United Global Convention on 4th August, 1947, Dr J.B. Danquah and his colleagues advocated for political pluralism, they advocated for the respect for human rights, they advocated for democratic accountability, and stood against the one-party state. Mr Speaker, today, if you want to appreciate the legacy and memory of Dr J.B. Danquah, look no further, just take a look at Article 3 (1) of our national constitution, where the framers of our national constitution, in responding to the efforts made by the likes of Dr J.B. Danquah against one-party state, have expressly prohibited the establishment of a one-party state. That is very much part of the legacy of Dr J.B. Danquah.

Mr Speaker, the case of *Re: Akoto and 7 Others [1961] GLR 523-535* continues to be a *locus classicus* when it comes to the right to fair trial, and the right of all citizens not to be condemned and not to be sentenced and not to be detained without a fair trial. This case is one which anybody who has had the privilege of passing through the law school would have come across it. It continues to be a *locus classicus*.

Mr Speaker, Dr J.B. Danquah's work, indeed, when he got into the Legislative Council of our country, was described by the Watson Commission as the "doyen of Gold Coast politics", where he spoke extensively and advocated extensively for the Ghana that is being modelled today. Mr Speaker, at the time, it was not fashionable; the Ghana of rule of law, the Ghana of respect for human rights, the Ghana of democratic accountability, the Ghana of individual rights and individual liberties, as well as individual enterprise. At the time when Dr J.B. Danquah made this case before the Watson Commission, that was not fashionable. But today, that is the Ghana that has been constructed. All he stood for at the time has become the pillars, foundations, and the bedrock of modern-day Ghanaian democratic experiment.

Mr Speaker, Dr J.B. Danquah did not only contribute to the politics of our country, he was also very instrumental when it came to intellectual contribution. Dr J.B. Danquah was very instrumental in the establishment of the University College of the Gold Coast, which is today the University of Ghana, and

indeed, Dr J.B. Danquah co-founded the Ghana Academy of Arts and Sciences. The Ghana Academy of Arts and Sciences has become the foremost platform for scholarship, critical thought, and national reflection, and it continues to serve our country.

Mr Speaker, Dr J.B. Danquah also contributed to the economic development of our country in terms of the blueprint for our national development, some of which continue to contribute and impact the forward march of our country and the national economic development of our country. By that, Mr Speaker, I refer to Dr J.B. Danquah's efforts in the mid-1940s, where he advocated for cocoa farmers, which culminated in the establishment of the Cocoa Marketing Board in 1947, today's COCOBOD.

Mr Speaker, if I may emphasise, today's Ghana Cocoa Board (COCOBOD), if we are to be blunt about it, indeed, arises out of the direct efforts of Dr J.B. Danquah in the mid-1940s, when he stood up and advocated and fought for cocoa farmers at the time, which culminated in the establishment of the Cocoa Marketing Board of 1947. This is what has given us COCOBOD today.

Mr Speaker, Dr J.B. Danquah did not end with intellectual interventions. He was also very instrumental when it came to the social and cultural development of our country. It is trite, and I need not point out, that even the name of our country, Ghana, was through the intellectual and scholarly research of Dr

J.B. Danquah, where he undertook extensive scholarly research into the then Ghana Empire, and made a proposal that, at the time, Gold Coast, which was on the verge of gaining independence, should be named Ghana. And so, Mr Speaker, I should say that Ghana, to some extent, owes its name to the tireless work and scholarly work of Dr J.B. Danquah.

Mr Speaker, I want to conclude by saying that if there was one legacy which stands in the name of Dr J.B. Danquah, I will say that that legacy is today's Ghana—

The Speaker: Hon Member, if you want to continue, the First Deputy Speaker will take the Chair, and then you can conclude. I had a few things to say, but I have to rush for another meeting.

4.11 p.m. —

[MR FIRST DEPUTY SPEAKER,
HON BERNARD AHIAFOR, IN THE
CHAIR]

Mr First Deputy Speaker: Hon Member, you are to conclude.

Mr Samuel A. Jinapor: That is so.

Mr Speaker, let me conclude on just two points: the first point, the construction of modern-day Ghana. Dr J.B. Danquah played a significant role. As the historians will tell us, at the time Ghana was to be birthed, the Asante colony was to be part of modern-day Ghana. It took the ingenuity, the diplomatic skills, and the efforts of Dr J.B. Danquah to lobby the then King of

Asante, His Royal Majesty Otumfuo Prempeh II, to convince him and bring the Asante colony on board to make today's Ghana what it is. That was Dr J.B. Danquah's role.

4:15 p.m.

Mr Speaker, finally, if there was one legacy that is attributed to Dr J. B. Danquah, I would say it is the nature of today's Ghana because what J. B. Danquah stood for, before our independence, during our independence, after our independence, and indeed, I dare say, what he died for in Nsawam prison, are essentially what constitutes today's Ghana. I refer to J. B. Danquah's tireless advocacy, fight and effort to promote rule of law, to fight against one party state, to advocate for democratic accountability, to advocate for a unitary Ghana, a one Ghana, where we are all one people, the respect for human rights, the promotion of individual, individual and private enterprise in Ghana. That is the Ghana we have today, and that is the Ghana we have all built a consensus around, which is a Ghana where we have political pluralism, democratic accountability, respect for human rights, rule of law, and private enterprise. That is the Ghana we have today; that is the Ghana J. B. Danquah fought for in the 40s, the 50s and the 60s. That is the Ghana J. B. Danquah died for. And that is the Ghana for which he was misunderstood then, but that is the Ghana we have today.

Mr Speaker, let me conclude by saying that Dr Joseph Kwame Kyeretwie Boakye Danquah

indeed came, he fought, he advocated, he worked for his country, and his legacy continues to shine.

Mr Speaker, with these words, I thank you for the opportunity.

Mr First Deputy Speaker: Yes, Hon Fiifi Buckman?

Mr Philip Fiifi Buckman (NDC — Kwesimintsim): Mr Speaker, thank you very much. I want to add my voice to earlier speeches made.

Mr Speaker, indeed, it is true that J. B. Danquah was an intellectual. It is true he was a historian. It is true he was a lawyer, and I must say, he was also a prophet. He was a prophet because he told the gathering at Saltpond that if all of them failed Ghanaians, Nkrumah was not going to fail them. [*Hear! Hear!*] Indeed, when they failed Ghanaians, Nkrumah stood up and saved Ghana.

Mr Speaker, I want to also add that indeed, as a historian, he was touted as the doyen of Ghana's history. J. B. Danquah, indeed, before parting ways with Nkrumah ideologically, came out with the name Ghana, which was adopted, which is true, because he traced Ghana from the ancient Ghana Empire. And that was what they were all thinking about until they fell apart. So, with the characteristics that Ghana shared with the ancient Ghana Empire, this current Ghana was named Ghana because of what he proposed. But as I said, we have come this far by the struggles of our leaders. And it should dawn on us that we should not only cherish people according

to their political sides, but whoever contributes to the betterment and upliftment of this nation; we should celebrate that person. And it is on that score that I support the celebration of J. B. Danquah.

But, Mr Speaker, in the United Coast Convention (UGCC), there was this gentleman called Paa Grant, who was the financier of the UGCC. He was an Nzema from Sekondi [sic], who has to also be celebrated. So, I would call upon my Brothers that even as they celebrate J. B. Danquah, even as they celebrate people that they cherish, they should also celebrate the United Gold Coast Convention's financier, who was Dr Paa Grant. [*Hear! Hear!*]

Thank you very much, Mr Speaker.

Mr First Deputy Speaker: Thank you, Hon Member.

Yes, Hon Kojo Oppong Nkrumah?

Mr Kojo Oppong Nkrumah: Mr Speaker, thank you for the opportunity to contribute to this Statement. I want to join in commending the Member of Parliament for Abuakwa South, Hon Kingsley Agyemang, for this commemorative Statement. Indeed, himself and his predecessors have always reminded the nation and this House about the great work that Joseph Kwame Kyeretwie Boakye Danquah did for the Republic of Ghana and has contributed to our nation's progress.

Mr Speaker, I come from a cocoa-growing village, Akyem-Anyinase in the Ofoase Ayirebi Constituency. And on a

day like this, as we commemorate the memory of J. B. Danquah, for people in the cocoa industry, it invokes a lot of interesting memories. Because, J. B. Danquah contributed a lot to the growth of the cocoa industry in Ghana. It will be recalled, as has been mentioned by some of my Colleagues earlier, that he was the chief advocate for the setting up of the Cocoa Marketing Board, an entity known today as Cocoa Board, responsible for organising the framework around which the cocoa industry can be expanded, and cocoa farmers can be supported.

Today, at a time when cocoa farmers are weeping because of the difficulties that the sector and this institution is experiencing, the work that J. B. Danquah and people like him did in times past comes into stark focus. It will be recalled that around the 1940s, he was again one of the strongest voices that the Local Buying Companies (LBCs), should not be made up of foreigners. It should be local companies, local individuals, responsible for leading the exercise of buying cocoa from our cocoa farmers. Indeed, when the colonial government wanted to introduce a programme to cut down cocoa trees because of the swelling shoots around the time, he was one of the strongest voices who was advocating that alternative methods should be employed. When it came to education, he was one of the strong voices advocating that funding from cocoa be made available for the establishment of some of our universities. Indeed, it went further to influence the provision of cocoa scholarships to the sons and daughters of cocoa farmers.

Today, as we commemorate his memory and his work and his contribution to the cocoa industry, for the many in the cocoa enclaves, there is worry that all this great work that was laid out there by people like J. B. Danquah is now coming to naught. As for months, cocoa that has been grown and harvested through the toil and sweat of our farmers has not even been paid for by Cocoa Board and the LBCs that are out there. And so, Mr Speaker, we should not only just celebrate and commemorate his memory, the necessary steps that should be taken to ensure that true life is given to the things that he advocated for must be put in place.

Mr Speaker, I thank you for the opportunity to have contributed to this work.

Mr Annoh-Dompreh: Mr Speaker, let me commend Dr Kingsley Agyemang, who definitely inherited the ritual which was dutifully performed over the years by Hon Atta Akyea, in making a commemorative Statement in respect of our thought bearers that respected the doyen of Ghana politics, Dr. J. B. Danquah.

4:25 p.m.

Mr Speaker, earlier speakers have spoken to various narratives I consider important, and hence, I intend to limit myself to just a few concerns. It is clear that J. B. Danquah bridged the gap between the downtrodden farmers and the big people in society. He was counsel for farmers and the evidence is all over in terms of what he achieved eventually,

advocating for the welfare of the farmer. Colleagues have spoken about cocoa farmers. I think it goes beyond cocoa farmers. The start of the advocacy was mainly about the welfare of the Ghanaian farmer. The establishment of the Ghana Cocoa Board (COCOBOD) was just one of the many achievements and progress he chalked up in advocating for the welfare of farmers.

Protection of our lands against our colonial masters, the Aborigines' Rights Protection Society, and the like. The man has distinguished himself. Oftentimes, we yield to this tendency of drawing a wedge between J. B. Danquah and Kwame Nkrumah, which in my view is totally unnecessary. Suffice it to say that they had different views in terms of how the country should be governed. They made significant achievements in their own right. Hence, it is important that we avoid this tendency to always draw comparisons. It is totally unnecessary.

I was struck by the apprehension and even condemnation when a call to name the University of Ghana after J. B. Danquah was raised. It saddened my heart that such a personality would be treated the way he was treated. But I also concede that the remembrance of our forbearers should not reflect in monuments per se. Elsewhere in other developed jurisdictions, the Americas, Britain, and Australia, they distil the principles and the things they stood for. These are taught in schools and institutions, and they become part of their body politics and the formation of those institutions. For me, that should be the trust of the argument. But it was sad that we received these apprehensions in

the call for the University of Ghana to be named after him. I think the Ghana Tourism Authority could make some amendments in terms of how we can cash in on the legacies of J. B. Danquah.

I am surprised that up to this stage, even in the cells where he gave up the ghost, the Ghana Tourism Authority has not cashed in on that. It is something that we can cash in on. People will come from near and far to see these. The site where he was buried must be developed by the Ghana Tourism Authority to become a point of attraction. His tabloids are all over. Go to the University of Ghana, the Balme Library, and one could read a lot of wonderful interventions by J. B. Danquah. But I think that the remembrance of such a personality should rather be a binding force that will unite us as a country. I get sad when Colleagues and Ghanaians tend to drive this wedge. It is totally unnecessary. Let us celebrate these forbearers in their own right and the things that they were able to achieve for our country.

There is a roundabout named after J. B. Danquah. I think this is not enough. We can go beyond that. And for me, that is the trust we are talking about. The Preventive Detention Act (PDA), the development of our constitutional jurisdiction, and all his contributions must be distilled and celebrated. That is what we should be doing as a country. I think that if we continue to do that, we will leave a good legacy for this country and for generations yet to come.

Mr Speaker, not to sound repetitive, I want to thank you for the opportunity and to commend once again the maker of the Statement for such brilliant write up.

Mr First Deputy Speaker: Thank you very much.

Hon Members, Statement time is ended. Majority Leader, any indication?

Mr Ayariga: Mr Speaker, are you asking whether I want to contribute to the commemorative Statement?

Mr First Deputy Speaker: Statement Time is ended.

Mr Ayariga: All right.

Mr Speaker, for so many years I have had this lecture in this Chamber. I think I am well-schooled, so I will not say much.

Mr Speaker, we will take item numbered 7(a), a number of Papers that need to be laid.

Mr First Deputy Speaker: Very well.

Yes, Minority Chief Whip?

Mr Annoh-Dompreh: Mr Speaker, will the Leader come clear? Is he laying all the items advertised under 7(a), or are there specific ones he wishes to lay?

Mr Ayariga: Mr Speaker, I am laying item numbered 7(a), I to V. Then the Minister of State in charge of Government Communication will lay (b). Then the Chairperson of two Committees will lay two other Reports, but those are not ready.

So let us focus on the item numbered 7(a) and leave out (b), (c), and (d).

Mr First Deputy Speaker: Hon Members, at the commencement of Public Business, Presentation of Papers, item numbered 7(a) by the Majority Leader.

PAPERS

By the Majority Leader —

Report of the Auditor-General on the Statements of Foreign Exchange Receipts and Payments of the Bank of Ghana (BoG) for the Half-Year Ended 30th June 2025.

Referred to the Public Accounts Committee.

Medium-Term National Development Policy Framework (2025-2029).

The Coordinated Programme of Economic and Social Development Policies (2025 – 2029).

Ghana National Action Plan on Business and Human Right.

Referred to the Committee on Economy and Development.

Mr First Deputy Speaker: Yes, Hon Member?

Mr Annoh-Dompreh: Mr Speaker, may I draw your attention to the referral you just made? Ghana National Action Plan on Business and Human Right. I think the Human Rights Committee should have an interface. Either you would want to do a joint, or we could refer it to the Committee on Economy

and Development and include the leadership of the Committee on Human Rights.

Mr First Deputy Speaker: Very well. The leadership of the Committee on Human Rights should join the Economic and Development Committee to consider and report.

By the Majority Leader—

Report of the Parliamentary Delegation on the 150th Inter Parliamentary Union Assembly held in Tashkent, Uzbekistan.

Mr First Deputy Speaker: Yes, Majority Leader.

4:35 p.m.

Mr Ayariga: Mr Speaker, we also have *Order Paper Addendum* and something that Hon Kojo Oppong Nkrumah has been advocating, and we have so dutifully listened and responded to him; it is the very comprehensive policy document on the 24-Hour Economy. So, the 24-Hour Economy and Accelerated Export Development Programme, based on a request that has been made by my Friends in the Minority. So, Mr Speaker, the object of the *Order Paper Addendum* is to create space for this Report to be laid so that it can be shared among Hon Members of the House.

So, Mr Speaker, with your kind permission, if we could go to the *Order Paper Addendum*, the item numbered 1(a).

Mr First Deputy Speaker: Yes, Hon Minority Chief Whip?

Mr Annoh-Dompreh: Mr Speaker, we on this Side of the House, as the Hon Majority Leader knows, we will not object to the laying of Papers; however, the Hon Leader prefixed his application with a fact that “as a result of the call of the Minority”, and that is very revealing. So, indeed, if the Minority had not intervened, they would have been sleeping, and that is very interesting

So, Mr Speaker, we are not objecting—

Mr First Deputy Speaker: Hon Minority Chief Whip, the House is made up of the Majority and the Minority.

Mr Annoh-Dompreh: So, he should not be in a hurry to celebrate. We are yet to peruse the content of the document. I can see the document is colourful. As to whether that colourful nature of the document is expressed in the efficiency of the content of that document, that is another matter.

So, Mr Speaker, he can proceed, and we would not object.

Mr First Deputy Speaker: Hon Members, the item numbered 1(a) on the *Order Paper Addendum* by the Hon Majority Leader.

By the Majority Leader —

The 24-Hour Economy and Accelerated Export Development Programme.

Referred to the Committee on Economy and Development.

Yes, Hon Majority Leader, are we taking the item numbered 1 (b)?

Mr Ayariga: There is an appeal for the Leadership of the Committee on Trade, Industry and Tourism to join the Committee on Economy and Development.

Mr First Deputy Speaker: Very well. I will accede to the request. Leadership of the Committee on Trade, Industry and Tourism should accordingly join the Committee on Economy and Development.

Mr Annoh-Dompreh: Mr Speaker, the practice has been that any time these applications are made to you, you refer to our Side of the Leadership for our input, but you did not.

Mr First Deputy Speaker: Do you have a contrary view?

Mr Annoh-Dompreh: No, I do not.

Mr First Deputy Speaker: Very well.

Do we take the item numbered 1(b)?

Mr Ayariga: Yes, Mr Speaker. The item numbered 1(b) is the Report of the Committee on Constitutional and Legal Affairs on the 24-Hour Economy Authority Bill.

Mr Speaker, it is not just a report on the policy or policy document that has

been laid, but the Bill itself to establish the Authority was presented to this House, referred to your Committee, the Committee has worked on it, and the Committee is ready to lay the Report.

Mr Speaker, we can go to the item numbered 1(b).

Mr First Deputy Speaker: Yes, Hon Minority Chief Whip?

Mr Annoh-Dompreh: Mr Speaker, we are watching with an eagle eye.

Mr First Deputy Speaker: Very well.

The item numbered 1(b) on the *Order Paper Addendum*, by the Chairperson of the Committee.

Mr Enu: Mr Speaker—

Mr First Deputy Speaker: Hon Member, are you the Chairman of the Committee?

Mr Ayariga: The Chairperson, I think, has stepped out, so that is the Vice Chairperson of the Committee.

Mr First Deputy Speaker: Very well. Vice Chairperson of the Committee, you may do so.

By the Vice Chairman of the Committee—

Report of the Committee on Constitutional and Legal Affairs on the 24-Hour Economy Authority Bill, 2025.

Mr First Deputy Speaker: Yes, Hon Majority Leader, any indication?

Mr Ayariga: Mr Speaker, I do not think we have any further Business for the day.

Mr Speaker, I think we can call it a day.

Mr First Deputy Speaker: Yes, Hon Minority Chief Whip?

Mr Annoh-Dompreh: Mr Speaker, the Hon Leader of the House a while ago made an application that we should draw the curtains. I just want to remind the Hon Leader of the House that we just laid a Bill on the 24-Hour Economy. Besides the point, we started Sitting at 2 p.m. We have Businesses scheduled on the *Order Paper*. Why do they want to run and go home? For instance, we have the Report of the Committee of Independent Constitutional Bodies; their work is done, and their Report is ready. Also, the Report of the Committee on Security and Intelligence on the Security and Intelligence Agencies Bill; their Report is done for Second Consideration, and they want to run and go home, yet they have laid a Bill on the 24-Hour Economy. What is that? We are not going home. We want to work. How many hours have we done? Just two hours.

4:45 p.m.

Mr Speaker, we are opposed to going home. We have done just two hours and they want to run home. Mr Speaker, Ghanaians are watching them. They have

laid a 24-hour Economy Bill, and they only work for two hours today. They have a work schedule which is outstanding, yet they want to run home. How ironic? Mr Speaker, we are opposed to any application by—

Mr First Deputy Speaker: Hon Minority Chief Whip, I beg to differ. The 24-hour Economy Bill has been laid long ago. What has been presented for distribution is a Report of the Committee. So, to say that we have just laid the 24-hour Economy Bill, for all intent and purposes, with all due respect, cannot be correct.

Mr Annoh-Dompreh: Mr Speaker, you know you are my very good Friend, and in these matters, I will always defer to you. Mr Speaker, I am essentially saying that we have scheduled Businesses. The Reports are ready. Can the Majority Leader tell this House why we cannot take items 7 (c) and (d)? They are ready. The Reports are ready. We have only worked for two hours today.

Mr First Deputy Speaker: Yes, so when the Reports are ready, the Reports are for distribution to Hon Members. That was what I said upon presentation of the Report.

Mr Annoh-Dompreh: Mr Speaker, with respect, I have done my checks, and the Reports are—

Majority Leader, sit down. Let me finish speaking.

Mr Speaker, the Reports are ready. Unfortunately, the Majority Chief Whip has abandoned him. Otherwise, the

Majority Chief Whip would have checked at the printing room. They would have realised that the Reports are ready. Meanwhile, he is alone. He stands alone. The Majority Chief Whip has abandoned him.

Mr First Deputy Speaker: The rules are clear that when sufficient copies are ready, it is due for presentation. So, I agree with you that sufficient copies are ready. It is being presented, and it is now going to be distributed to Hon Members.

Yes, Majority Leader?

Mr Ayariga: Mr Speaker, I just had a meeting with the Chairperson and Ranking Member of the Committee on Security and Intelligence. I was hoping that they would be able to lay their Report. But they indicated that so many interest groups sent memoranda. So, I agreed with them on a timetable and gave them up to Monday to have a meeting with the various interest groups that have submitted memoranda to them and then incorporate the views of those interest groups into the Report before it is laid. The reason is that we do not want those interest groups to feel that we did not listen to them. I even recommended that the hearings where the interest groups will be invited should be televised. So, the Report is technically not fully ready. That is why I am not asking for the laying of the Report.

If the other Side actually wants, I can lay item numbered (d). Actually, the First Deputy Speaker and I undertook the visits to the Independent Constitutional Bodies, and that Report is in fact available. So, we can lay that one.

Mr First Deputy Speaker: Very well. Item numbered 7 (d), Chairperson of the Committee?

PAPER

By the Chairperson of the Committee —

Report of the Committee on Independent Constitutional Bodies on a familiarisation visit to Six (6) Independent Constitutional Bodies.

Mr Mahama Ayariga: Mr Speaker, I move that this House adjourns to 2 p.m. tomorrow in the afternoon.

Mr First Deputy Speaker: Hon Members, Motion moved for adjournment. Any seconder?

Alhaji Habib Iddrisu: Mr Speaker, I second the Motion.

Mr First Deputy Speaker: Hon Members, before working on the Motion, let me crave your indulgence to announce the presence of visitors from South Korea, members of the Korean Industrial Centre.

First on my list is Mr Hongkoo Hwang (Director KOTRA), Mr Youngtae Choi (CEO MK Pollex), Mr J Lee (CEO Yiyu Korea), Mr Minjae Kim (Vice CEO Samwon), Mr Tom Yoo (CEO, TTS), Mr Sanghoon Lee (CEO, Hyun Engineering), Mr Jiyeon Choi (CEO, Kyung Dong), and Mr Joohyung An (CEO, CEDAR). [*Hear! Hear!*]

[Members of the Korean Industrial Center were acknowledged]

Hon Members, on your behalf, I welcome them to Parliament of the Republic of Ghana. Hon Members, may you have a fruitful visit to Parliament of Ghana.

Hon Members, Motion has been moved and seconded for adjournment of the House.

Question put and Motion agreed to.

ADJOURNMENT

The House was accordingly adjourned at 4:51 p.m. till Thursday, 5th February, 2026 at 2:00 p.m.

